Riverside General Hospital

DISCHARGE SUMMARY

Patient:	John A. Doe	MRN:	00482917
DOB:	03/14/1958 (68 y)	Sex:	Male
Admission Date:	10/21/2026	Discharge Date:	10/26/2026
Attending:	Dr. Sarah Mitchell, MD	Service:	Internal Medicine
Length of Stay:	5 days	Disposition:	Home with self-care

Chief Complaint

Substernal chest pressure and shortness of breath on exertion for two days, with one episode of lightheadedness on the morning of presentation.

Past Medical History

Past Medical History: Hypertension, Type 2 Diabetes. No prior history of myocardial infarction. No history of stroke, heart failure, or chronic kidney disease. Hyperlipidemia diagnosed 2019.

Social and Family History

Former smoker (15 pack-years, quit 2012). Drinks alcohol occasionally (1-2 drinks per week). Retired school administrator, lives with spouse. Father had coronary artery disease at age 66; mother had Type 2 Diabetes.

Allergies

Penicillin (rash). No other known drug allergies.

Hospital Course

Mr. Doe presented to the Emergency Department with exertional chest pressure and dyspnea. Initial ECG showed normal sinus rhythm at 78 bpm with nonspecific T-wave flattening in the lateral leads and no ST-segment elevation. Serial high-sensitivity troponin I values were 8, 9, and 8 ng/L (reference <14 ng/L), effectively ruling out acute myocardial infarction.

He was admitted to telemetry for observation. No arrhythmias were recorded. A transthoracic echocardiogram demonstrated a left ventricular ejection fraction of 58% with mild concentric left ventricular hypertrophy and no significant valvular disease. Cardiology was consulted and recommended a pharmacologic stress test, which was negative for inducible ischemia (see separate Cardiology Report).

Blood pressure on admission was 158/94 mmHg and was optimized with an increased lisinopril dose. Glycemic control was reviewed; admission HbA1c was 7.4%. The chest discomfort was ultimately attributed to hypertensive urgency with demand-related symptoms and possible musculoskeletal contribution. The patient remained chest-pain free for the final 72 hours of admission.

Procedures and Key Results

Test	Result
ECG	NSR 78 bpm, nonspecific lateral T-wave flattening, no ST elevation
hs-Troponin I (x3)	8, 9, 8 ng/L (normal)
Echocardiogram	LVEF 58%, mild concentric LVH, no significant valve disease
Pharmacologic stress test	Negative for inducible ischemia
HbA1c	7.4%
Creatinine / eGFR	1.0 mg/dL / 82 mL/min/1.73 m2
LDL cholesterol	138 mg/dL

Discharge Diagnoses

1. Hypertensive urgency, resolved
2. Chest pain, non-cardiac / low-risk after evaluation
3. Type 2 Diabetes Mellitus
4. Hyperlipidemia

Discharge Medications

Discharge Medications: 1. Lisinopril 10 mg PO daily

#	Medication	Dose	Route	Frequency	Notes
1	Lisinopril	10 mg	PO	Daily	Blood pressure; dose increased this admission
2	Atorvastatin	40 mg	PO	Daily at bedtime	Hyperlipidemia; continued
3	Metformin	500 mg	PO	BID with meals	Type 2 Diabetes; continued

No new medications were started other than the dose adjustment above. Aspirin 81 mg was not prescribed given low-risk evaluation; this may be revisited by the primary care provider. Avoid penicillin-class antibiotics (documented allergy).

Medication Changes Summary

Changed: Lisinopril increased from 5 mg to 10 mg daily.
Continued: Atorvastatin 40 mg, Metformin 500 mg.
Stopped: None.

Condition at Discharge

Stable. Vitals at discharge: BP 128/78 mmHg, HR 72 bpm, SpO2 98% on room air, afebrile. Ambulating independently; alert and oriented.

Plan & Follow-up

Plan & Follow-up: Continue Atorvastatin 40 mg daily at bedtime. Continue Metformin 500 mg twice daily with meals and Lisinopril 10 mg daily.

Appointment	Timeframe	Purpose
Primary care (Dr. Alan Reyes)	Within 1 week	BP check, basic metabolic panel (potassium, creatinine)
Cardiology (Dr. Priya Nair)	In 4 weeks	Review of stress test and echocardiogram; risk-factor management
Endocrinology / diabetes educator	In 6-8 weeks	Glycemic review; HbA1c goal below 7%

Patient Instructions

- Follow a low-sodium (<2 g/day), heart-healthy, carbohydrate-consistent diet.
- Check blood pressure at home daily and keep a log; bring it to your appointments.
- Walk 30 minutes most days as tolerated.
- Do not stop medications without speaking to your doctor.

Return Precautions

Return to the Emergency Department or call 911 for chest pain or pressure lasting more than 10 minutes, shortness of breath, fainting, new weakness or speech difficulty, blood pressure above 180/110 mmHg, or swelling of the lips or face (possible ACE-inhibitor reaction).

Electronically signed: Sarah Mitchell, MD - 10/26/2026 16:42

This is a fictional, synthetically generated document for software testing. It does not describe a real person.